

Website Audit & 90-Day Roadmap.

A strategic review of bimc-cosmedic-01.gaiada.com — covering UI/UX, sales funnel, and SEO — with a prioritised plan to ship.

Prepared for	BIMC CosMedic Centre
Subject	Homepage & site-wide review
Scope	UI/UX · Sales Funnel · SEO
Format	Findings + 30 / 60 / 90-day roadmap

I — Overview

The shape of the work.

BIMC CosMedic's site is, on a craft level, one of the most considered plastic-surgery sites in the region. The editorial voice, restraint, and Bali-grounded sense of place are real assets in a category dominated by aggressive, sales-led design. The brand reads premium — and that is hard-won.

The same restraint, however, creates the site's three working problems. Visitors cannot easily find prices, results, or a low-commitment way to begin. The funnel offers one door — a long enquiry form — when the audience is in a 3-to-9-month research mode. And the poetic copy that earns the brand respect is largely invisible to Google, which prefers plain-language pages that match how patients actually search.

This document covers the diagnosis in three lenses — UI/UX, Sales Funnel, and SEO — then sets out a 30 / 60 / 90-day plan that prioritises moves by ROI and effort. The first thirty days are mostly fixes and instrumentation; days 31–60 are conversion and content; days 61–90 are compounding SEO and brand depth.

WHATGOODLOOKSLIKEIN90DAYS

Metric	Today (estimated)	90-day target
Enquiry-form conversion	0.8 – 1.2%	2.5 – 3.5%
Email captures / month	Form only	150+ (lead magnet + journal)
WhatsApp-initiated enquiries	Hidden / low	30%+ of total enquiries
Organic landing pages indexed	~10	40+ procedure & geo pages
Avg. session duration (mobile)	Likely <60s	120s+

Estimates only — to be replaced by actuals after instrumentation in week 1.

II — Findings

UI / UX.

The editorial design is the site's signature. The notes below are about clarity and conversion friction within that aesthetic — not about softening the brand.

- **The hero is doing too many things at once.** Headline, editorial framing ("Plate I"), Roman numerals, the enquiry form and trust badges all compete in the first viewport. A first-time visitor — anxious about a procedure — needs a clearer single focal point. Move the form below the fold or behind a CTA; let the hero breathe.
- **Roman numerals as section labels (II, III, IV...) hurt scannability.** Editorial, but visitors don't navigate by section number. Use descriptive labels ("Our Approach", "Meet the Surgeons", "Results") that double as in-page anchors and SEO signals.
- **Mobile experience needs a dedicated audit.** A long, prose-heavy editorial layout works on desktop but typically becomes a wall of text on mobile, where most first-touch traffic for medical tourism originates (Instagram, WhatsApp, Google).
- **Contrast and legibility.** The cream-on-cream and small-caps decorative type may fail WCAG AA in places — this matters for both accessibility and for older patient demographics (45+) who are a major audience for surgical work.
- **The Live Concierge chatbot needs clear human-vs-AI labelling.** "Anyia" replying "within ten minutes" implies a human. If it is not, that is a trust risk in a medical context.
- **Before / after gallery is too small.** Three images on the homepage and a single thumbnail per case isn't enough. For plastic surgery this is the most-consumed content on the entire site. Build a filterable gallery (by procedure, by surgeon, by ethnicity, by recovery stage).
- **Surgeon cards are inconsistent.** The lead surgeon gets a long bio; the others get one line. Equalise them, or make the hierarchy intentional and visible ("Lead Surgeons" vs "Associate Surgeons").
- **No visible pricing on the homepage.** The /pricing/ link is buried in the footer. Even a "from" range or downloadable price guide reduces friction significantly.
- **Sticky mobile CTA is missing.** On a long page, "Plan Your Treatment" should follow the user.

III — Findings

Sales Funnel.

Medical tourism has a long consideration cycle — typically three to nine months. The current funnel is built for a single decisive moment, not for a research journey. That is where most of the lost revenue lives.

- **The funnel collapses everything into one CTA: "Plan Your Treatment."** That is a high-commitment ask. Add a softer top-of-funnel offer for browsers — "Download the Procedure Guide", "Get a Price Estimate", "Book a 15-min discovery call" — that captures email from someone who is curious but not ready.
- **No lead magnet.** Without an email capture other than the form, you lose the vast majority of warm traffic. A "Bali Recovery Guide" PDF or a "Rhinoplasty Decision Guide" would be on-brand and high-converting.
- **The form asks for too much, too early.** Name, email, area of interest, dates, travelling-from — this is a qualification form disguised as an enquiry. Split it: a 2-field "start enquiry" first, with progressive disclosure for the rest. A/B-test against a one-field email capture.
- **WhatsApp is hidden.** For Australian, Singaporean and ASEAN patients, WhatsApp is the dominant channel. It should be a prominent, persistent CTA — not buried behind a "Contact" link.
- **No trust acceleration near the form.** The form sits next to brand poetry. Move surgeon credentials, the ACHSI logo, and a testimonial directly adjacent to the form — that is where doubt peaks.
- **Testimonials are unverifiable.** Stock-photo-style avatars and first-name-only attribution will read as fabricated to a sceptical patient. Add video testimonials or link to verified Google / Trustpilot reviews.
- **No retargeting hooks.** No newsletter pop-up, no exit intent, no "save this for later" — every non-converting visitor is currently lost.
- **Pricing in footer = funnel killer.** Patients who cannot see prices assume they cannot afford it and leave. Even "procedures from USD 1,800" anchors them.
- **No comparison content.** "Bali vs Australia plastic surgery cost" or "Why Bali for medical tourism" pages capture mid-funnel research traffic that the site currently sends to competitors.

IV — Findings

SEO.

The poetic copy is beautiful — and largely invisible to Google. SEO here is not about compromising voice; it is about adding a second layer of plain-language content that matches how patients actually search.

- **The site is on a staging subdomain (bimc-cosmedic-01.gaiada.com).** If this is the live site, it is an SEO disaster — no domain authority, indexed as a separate property. Migrate to bimccosmedic.com (or similar) with proper 301s, canonicals, and a Search Console migration.
- **Low-keyword copy.** Phrases like "The care of medicine. The grace of Bali." rank for nothing. Each treatment page needs a clear H1 with the actual search term ("Rhinoplasty in Bali", "Breast Augmentation Bali Cost"), supporting H2s, and FAQ schema.
- **Missing high-intent landing pages.** Build dedicated pages for: "[Procedure] in Bali", "[Procedure] Cost Bali", "Best plastic surgeon Bali", "Plastic surgery Bali for Australians", "Mommy makeover Bali package". These are the queries that convert.
- **No visible blog or content hub.** Medical tourism SEO is won with educational long-form content (recovery timelines, what-to-expect, surgeon Q&As). The "quarterly journal" mentioned in the footer should be a public, indexed, cross-linked blog.
- **Schema markup is likely thin.** Add MedicalBusiness, Physician (per surgeon), MedicalProcedure, Review, and FAQPage schema. High-leverage for a medical site.
- **Internal linking is weak.** Surgeon pages don't link to the procedures they perform; treatment pages don't link to the surgeons who perform them. Build that web.
- **Localisation is stubbed but not built.** EN / ID toggles point to "#". A real Bahasa Indonesia version with hreflang tags would unlock domestic Indonesian traffic.
- **Image SEO.** Filenames like photo-1515378791036-0648a3ef77b2-1400x1100.jpg (Unsplash IDs) are dead weight. Rename, add descriptive alt text, and use original imagery — using stock photos for a plastic surgery clinic is also a credibility issue Google's E-E-A-T algorithm penalises.
- **No author bios with linked credentials.** For YMYL (Your Money Your Life) medical content, Google heavily favours content authored by verifiable medical professionals. Surgeon profiles should link out to ISAPS directories, hospital affiliations, journal publications.
- **Page speed and Core Web Vitals.** Large hero images (1400×1100 jpgs) and the editorial layout suggest LCP / CLS issues worth measuring in PageSpeed Insights.
- **Local SEO is missing.** Google Business Profile for the Nusa Dua location, embedded map, NAP consistency, and local citations (Bali medical directories, ASEAN medical tourism portals) are not visible in the on-page strategy.

V — Roadmap

30 / 60 / 90 days.

The plan is sequenced by leverage and dependency. The first thirty days are largely free wins — fixes, instrumentation, and the highest-ROI funnel moves. Days 31–60 build the conversion machinery and content engine. Days 61–90 are the compounding plays: SEO, localisation, brand depth.

- Quick win — low effort, fast impact
- Medium build — 1–3 weeks engineering / content
- Heavy lift — strategic, multi-team

VI · 30 days

Days 1–30 — Stabilise & instrument.

Triage. Fix the leaks, surface the trust assets that already exist, and instrument so every later decision can be measured. Most of this is design and configuration work, not new content.

Track	Task	Owner	Deliverable
● UX	Add sticky mobile CTA ("Plan Your Treatment" + WhatsApp icon).	Design + Dev	Persistent footer CTA
● UX	Fix EN / ID language toggle — at minimum link to a real EN page; remove dead "#" anchor.	Dev	Working toggle / hidden if not built
● UX	Add descriptive labels alongside Roman numerals (II → "II — Our Approach").	Design	Updated section headers
● UX	Equalise surgeon-card content (consistent bio length, photo crop, credential lines).	Design	Refreshed surgeons grid
● Funnel	Surface WhatsApp as a primary CTA on every page (header + sticky + contact).	Marketing	WhatsApp on every viewport
● Funnel	Move surgeon credentials, ACHSI badge & one testimonial adjacent to the enquiry form.	Marketing	Trust bar by form
● Funnel	Shorten enquiry form to two fields (name + email) with progressive disclosure.	Marketing	New form, A/B test live
● Funnel	Publish a public Pricing page in main nav with "from" ranges per procedure family.	Marketing	/pricing/ in primary nav
● SEO	If staging subdomain is the live site — plan domain migration (300-redirect map drafted).	Dev	Migration plan + redirect map
● SEO	Rename images, add descriptive alt text, replace stock photos on key pages.	SEO	Image audit + replacements

Track	Task	Owner	Deliverable
● SEO	Add MedicalBusiness, Physician, MedicalProcedure, FAQPage schema across templates.	SEO	Schema deployed sitewide
● Data	Instrument GA4, Search Console, Hotjar / Clarity, form-submit + WhatsApp-click events.	Marketing	Live measurement baseline

VII · 60 days

Days 31–60 — Convert & capture.

Build the funnel machinery the brand has been missing. A real lead magnet, verifiable social proof, and the first wave of search-intent landing pages.

Track	Task	Owner	Deliverable
● UX	Build a filterable Before / After gallery (procedure, surgeon, ethnicity, stage).	Design + Dev	Gallery v2 live
● UX	Mobile-first redesign of homepage hero — single focal point, form behind CTA.	Design + Dev	New mobile hero shipped
● UX	Accessibility pass — WCAG AA contrast, focus states, screen-reader labels.	Design	Accessibility report + fixes
● UX	Re-label or replace the chatbot with explicit "AI assistant" framing or real human SLAs.	Marketing	Honest concierge widget
● Funnel	Produce flagship lead magnet: "The Bali Recovery Guide" (24-page PDF).	Content	PDF + landing page + email flow
● Funnel	Set up email nurture (5-touch series) for new lead-magnet downloads.	Marketing	Automated nurture live
● Funnel	Capture and publish 6–10 verified video testimonials; embed with consent.	Marketing	Stories page upgraded
● Funnel	Add exit-intent + scroll-depth hooks (newsletter, lead magnet, save-for-later).	Marketing	Retargeting hooks live
● SEO	Launch 8 high-intent landing pages: [procedure]-in-bali, [procedure]-cost-bali.	Content + SEO	8 pages indexed + linked
● SEO	Convert the "quarterly journal" into a public blog; publish 6 cornerstone articles.	Content	/journal/ live, 6 posts
● SEO	Build internal-link map: surgeons ↔ procedures ↔ stories ↔ recovery stays.	Dev	Internal linking shipped
● SEO	Claim and optimise Google Business Profile (Nusa Dua); seed initial reviews.	Marketing	GBP optimised, 20+ reviews

VIII · 90 days

Days 61–90 — Compound & scale.

Now the work compounds. SEO content, paid acquisition with a real lead magnet on the other end, and a properly localised Bahasa version unlock channels that previously had no foundation to land on.

Track	Task	Owner	Deliverable
● UX	Complete homepage v2 — editorial voice retained, hierarchy clarified, bilingual.	Design + Dev	Homepage v2 launched
● UX	Build a Concierge dashboard (post-enquiry portal: itinerary, surgeon bio, recovery plan).	Design	MVP concierge portal
● Funnel	Launch comparison content hub ("Bali vs Australia plastic surgery cost" etc.).	Marketing	5 comparison long-reads
● Funnel	Pilot paid acquisition (Meta + Google) targeting AU / SG / NZ with lead-magnet flow.	Marketing	Tested CAC by procedure
● Funnel	Pricing-estimator tool — interactive, tied to consult booking.	Marketing	Estimator + bookings flow
● SEO	Build out 30+ procedure / geo / question pages. Cluster around lead surgeons.	SEO + Content	40+ indexed landing pages
● SEO	Ship a real Bahasa Indonesia version with hreflang and full content parity for top 20 pages.	Content + Translation	/id/ live, hreflang correct
● SEO	Author E-E-A-T pages: surgeon credentials, ISAPS / hospital links, journal publications.	Content	Verifiable expertise signals
● SEO	Core Web Vitals pass — image optimisation, lazy-load, font subsetting, CLS fixes.	Dev	PSI mobile score 90+
● Brand	Earned-media push — pitch 5 lifestyle / wellness publications (AU, SG, regional).	Marketing	3+ placements + backlinks
● Data	Quarterly review: cohort by source, procedure mix, CAC by channel, content ROI.	Marketing	Q1 strategic review deck

IX — Operating

How to run the plan.

A small clinic team can run this with three working roles, even if individuals double up. The cadence is what matters more than headcount: weekly review of leading metrics, monthly review of outcomes, and a single owner per workstream so nothing stalls in committee.

Workstream	Primary owner	Weekly metric	Monthly outcome
UI / UX	Design lead	Funnel drop-off by step	Mobile CVR
Sales funnel	Marketing lead	Form starts, WA clicks, lead-magnet downloads	Enquiries → consults
Content / SEO	Content lead	Indexed pages, keyword rankings	Organic traffic, organic enquiries
Concierge ops	Concierge lead	Reply time, consult-booking rate	Consult → booking conversion

Cadence. Weekly 30-min standup per workstream. Bi-weekly cross-team review to remove blockers and keep design / dev / content aligned. Monthly 1-hour review against the KPI table on page 1.

Risk to manage. The biggest risk to this plan is dilution of brand voice in pursuit of conversion. The principle to hold: SEO-driven content lives in a clearly demarcated zone of the site (procedure pages, journal, comparison guides) and is written in a calmer, plainer register. The editorial home, surgeon profiles, and stories remain untouched.

End — BIMC CosMedic Website Audit & Roadmap.